# Before Stage 4 Planning: Recognizing and Referring Athletes with Mental Health Conditions

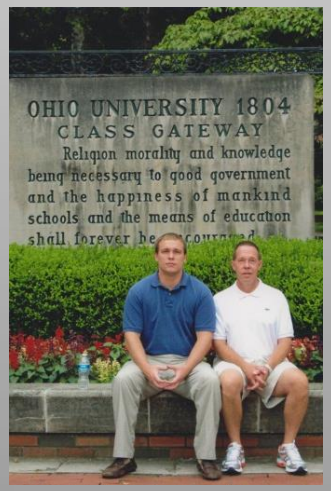
**Timothy Neal, MS, ATC**
**Chair, NATA Consensus Statements:**
**Recognition and Referral of Athletes with Psychological Concerns at the College Level (2013), Secondary School Level (2015)**

# An Athlete's "Worst Moment" May Not Always be a Physical Injury

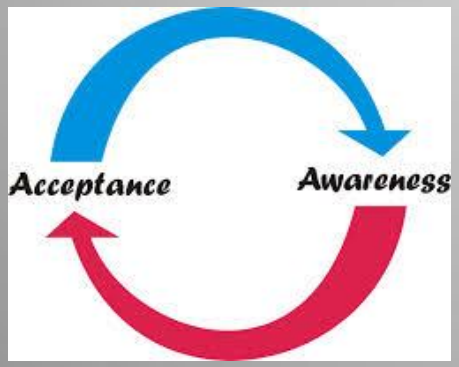
Postphoto by Jeff Hamilton

Dayton shortstop John Theis' ankle hung on mere ligaments after the collision with Scott Kuvinka.

# The Student-Athlete:
# More Than Just a Sports Participant



# Goals:
# Awareness, Recognize, Referral



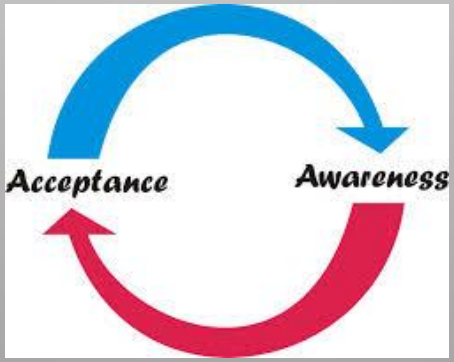
Awareness

Stigma

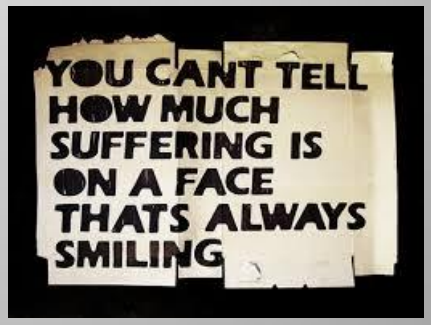
Prevalence

Stressors on Athletes

Situations that Challenge Mental Health



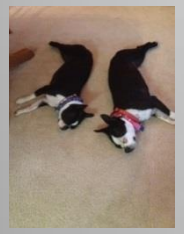
# Stigma Prevents Seeking Care

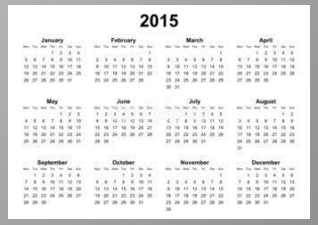


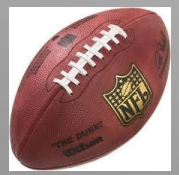
# Prevalence

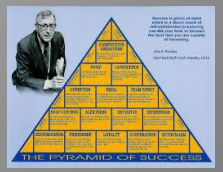
- One in every four to five adolescents and adults in America in the past year met criteria for a diagnosis for a mental health disorder.
- The rate of mental illness is more than twice as high in those in the 18-25 year old range (30%), than those aged 50 years and older (14%).
- Comorbidity of illnesses are found in 40% of those experiencing a mental disorder, compounding conditions and challenging care.
- Childhood adversities of parental loss, parental maltreatment, economic adversity raises prevalence to mental health disorders.
- Studies demonstrate that the majority of individuals with a mental health disorder never receive mental health care
(less than 25%- Columbia University study).
- Surveys report athletes hesitant to utilize counseling services
(less than 7% of injured athletes surveyed).

# Student-Athlete Expectations and Pressures









# Circumstances That May Impact an Athlete's Mental Health

- Concussions
- History of mental health issues
- ADHD
- Eating Disorders
- Overtraining
- Psychological challenge of injury

# Concussions

- Chemical change in brain
- Severity and length of recovery
- Return to learn, then return to play
- Monitor behaviors

# History of Mental Health Issues

- Depression
- Anxiety
- Eating disorders
- Intermittent Explosive Disorder
- Substance or alcohol abuse
- 6-17 year olds: 7.5% using medication for emotional or behavioral issues in U.S.

| Criteria for Presentations of Attention Deficit Hyperactivity Disorder (ADHD)* (modified from DSM-5) | | |
|---|---|---|
| **Inattentive** | Hyperactive-Impulsive | Combined Inattentive and Hyperactive-Impulsive |
| **Fails to give close attention to details or makes careless mistakes** | Fidgets with hands or feet or squirms in chair | Has symptoms of both inattentive and hyperactive-impulsive presentations |
| **Has difficulty sustaining attention** | Has difficulty remaining seated | |
| **Does not appear to listen** | Runs about or climbs excessively in children; extreme restlessness in adults | |
| **Struggles to follow through on instructions** | Difficulty engaging in activities quietly | |
| **Has difficulty with organization** | Acts as if driven by a motor; adults will often feel inside like they were driven by a motor | |
| **Avoids or dislikes tasks requiring a lot of thinking** | Talks excessively | |
| **Loses things** | Blurts out answers before questions have been completed | |
| **Is easily distracted** | Difficulty waiting or taking turns | |
| **Is forgetful in daily activities** | Interrupts or intrudes upon others | |

# Eating Disorders

- Focus on weight or appearance
- Over-exercise
- Drastic loss or gain of weight from past season
- Focus on caloric values of food
- Dehydrated
- Poor performance
- Suspect “anemia”

# Overtraining and the Athlete:
# 1. Culture of sport/coach
# 2. Pressure from family
# 3. Quest for success

- 100% effort expectation
- Year-round activity
- Lack of recovery physically and mentally
- Sleep disturbances
- Anxiety
- Depression

# The Psychological Challenge of Injury

- Feelings of frustration, anger, depression, uncertainty
- Disruption of routine; separation from team

- Threat to their identity as an athlete
- Fear of re-injury (48% for females, 21% for males)
- Season ending or career ending injuries
- Surveys of injured athletes reveal that they underestimated the emotional challenge of rehabilitation

# Recognition

Behaviors to Monitor

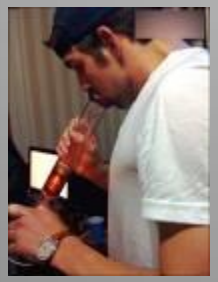
Suicide Ideation

Planning for the Worst

# Cat

Mental Health Disorders

Affect Mental Clarity and Behaviors



# Rare; Unexpected; Extreme Impact; and Retrospectively Predictable: **Suicide**

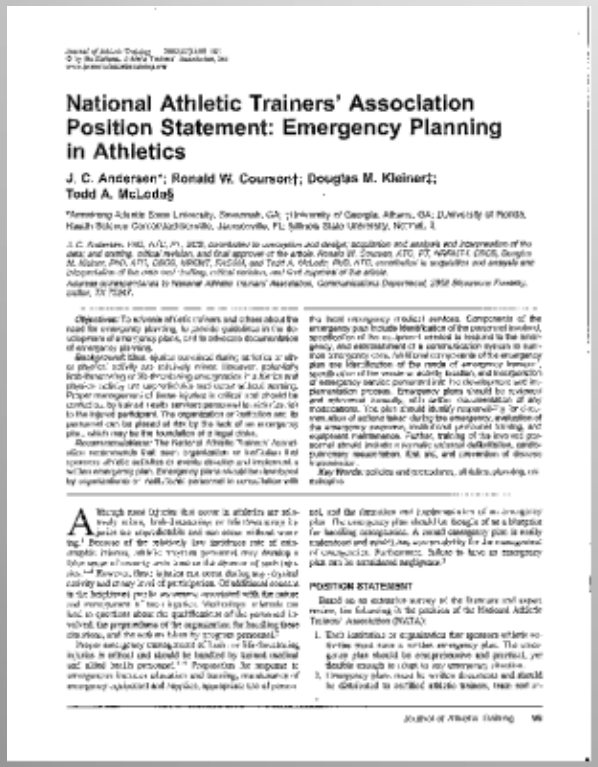
- Suicide is the third leading cause of death among NCAA student-athletes (NCAA study of student-athlete deaths, 2004-2008).
- The rate of suicide in the USA has been increasing since 2000; every 13.7 minutes, someone in the USA commits suicide.
- 1 in every 6 high school students SERIOUSLY consider attempting suicide annually in the U.S.
- 1 in every 13 high school students attempt suicide one or more times annually in the U.S.
- Survivors of loved ones that commit suicide should be observed for behaviors to monitor for assistance.

# T.I.P.A.:
## Thoughts, Intention, Plan, Access

- Are you having Thoughts of harming yourself?
- Do you Intend to harm yourself?
- What is your Plan to harm yourself?
- Do you have Access to things to harm yourself?

# Mental Health Incidents: Emergencies and Catastrophes

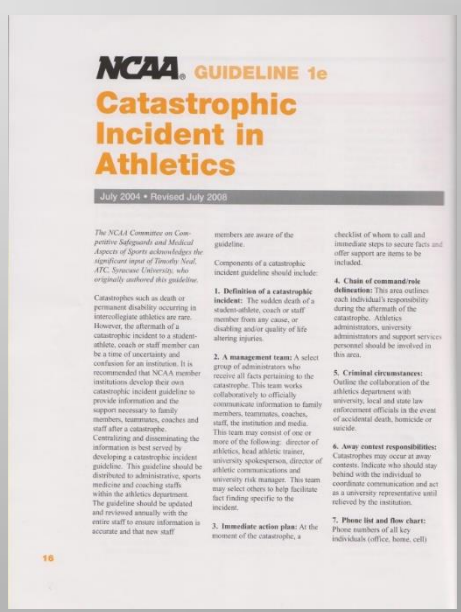
## Emergency Action Plan



National Athletic Trainers' Association Position Statement: Emergency Planning in Athletics

J. C. Andersen*; Ronald W. Courson†; Douglas M. Kleiner‡; Todd A. McLoda§

## Catastrophic Incident Guideline



NCAA GUIDELINE 1e

Catastrophic Incident in Athletics

July 2004 • Revised July 2008

# Referral

Approaching the Athlete with a Concern

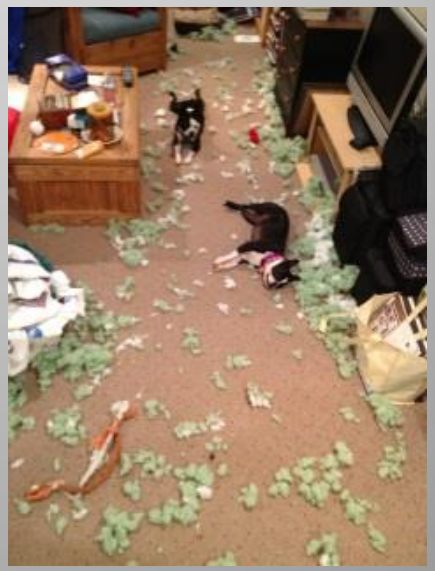
Confidentiality

Counseling

Collaboration

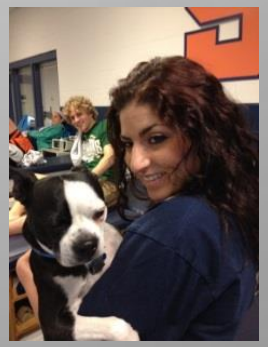
# Approaching the Athlete with a Potential Mental Health Issue

Letting a Sleeping Dog Lie vs. Fear of Waking it Up



# Approaching Questions

- How are things going for you? How is life going for you?
- Tell me what is going on.
- Your behavior (mention the incident or incidents) has me concerned for you. Can you tell me what is going on, or is there something I need to know why you behaved this way?
- Tell me more (about the incident).
- How did those cuts get there?
- Perhaps you would like to talk to someone about this issue?



# Confidentiality

- Respect the confidentiality wishes of an adult age athlete. Code of Conduct Violations permit contact of family and school authorities.
- Use of insurance for outside mental health care-alert EOB.
- The secondary school athlete is a MINOR; be sure to know school district and state laws are in confidentiality and mandated reporting protocols. Be sure the proper forms are utilized. Seek assistance if unsure of procedure.
- Point out that coaches and parents care about the student-athlete and it is helpful for them to understand.
- Use care if asked about a student- error on side of confidentiality

# Counseling

- Education on the psychological concern.
- Help identify “triggers” that may bring on or worsen the psychological condition.
- Provide information and encourage coping skills to address condition and “triggers” to help the patient become functional and enjoy life in spite of psychological concern.

# Referral Situations

- ROUTINE: help student-athlete make initial appointment

- SELF REFERRAL: encourage participation in counseling

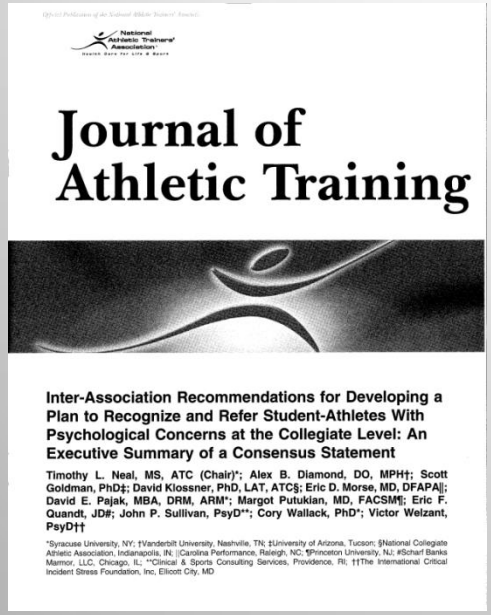
- EMERGENT REFERRAL: follow institutional protocol

1. do not leave student-athlete alone
2. accompany the student-athlete to facility directed to for assistance
3. communicate with administration, coaches
4. check with Student Affairs or institutional protocol on calling parents

- Goal is to identify and help a ISSUE, not react to an INCIDENT

# Collaboration

- Develop plan: school and community.
- Approval of risk management and general counsel.
- Talk about the importance of psychological health to students.
- “Stay in your lane”: only credentialed mental health care professionals are to diagnose and treat mental health disorders.



Journal of Athletic Training

Inter-Association Recommendations for Developing a Plan to Recognize and Refer Student-Athletes With Psychological Concerns at the Collegiate Level: An Executive Summary of a Consensus Statement

Timothy L. Neal, MS, ATC (Chair)*; Alex B. Diamond, DO, MPH†; Scott Goldman, PhD‡; David Klossner, PhD, LAT, ATC§; Eric D. Morse, MD, DFAPA||; David E. Pajak, MBA, DRM, ARM#; Margot Putukian, MD, FACSM¶; Eric F. Quandt, JD#; John P. Sullivan, PsyD**; Cory Wallack, PhD*; Victor Welzant, PsyD††

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***Journal of Athletic Training*** 2015;50(3):231–249
doi: 10.4085/1062-6050-50.3.03
_ by the National Athletic Trainers' Association, Inc

www.natajournals.org

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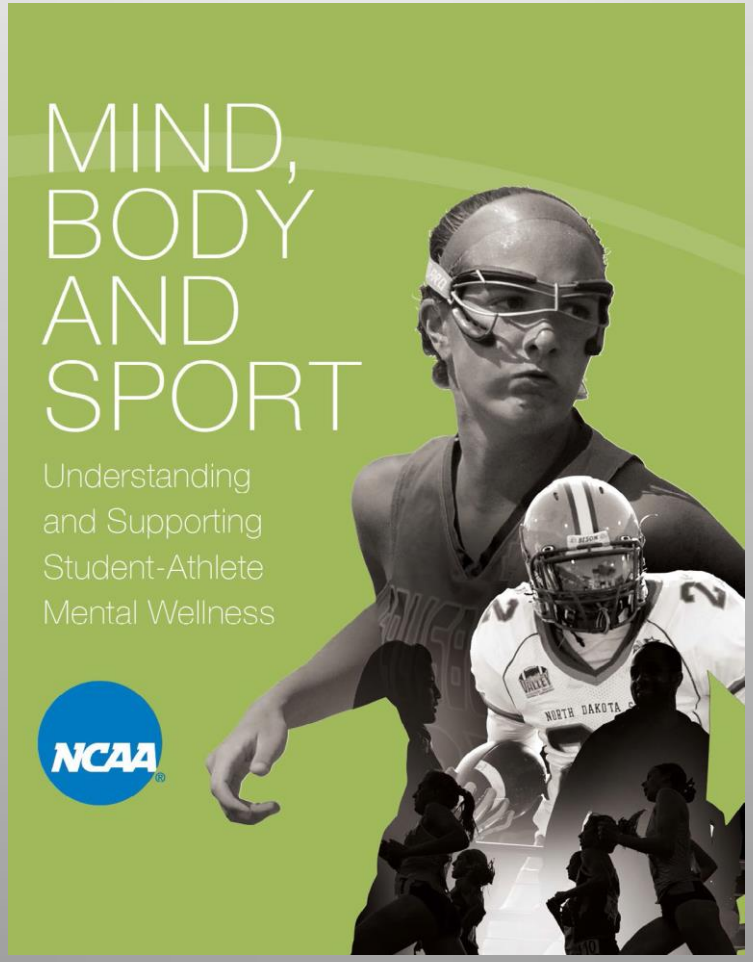
# NATA Consensus Statements

## Inter-Association Recommendations for Developing a Plan to Recognize and Refer Student-Athletes With Psychological Concerns at the Collegiate Level & Secondary School Level

www.nata.org/sites/default/files/psychologicalreferral.pdf

# NCAA


MIND,
BODY
AND
SPORT
Understanding
and Supporting
Student-Athlete
Mental Wellness
NCAA
NORTH DAKOTA

MHA
Mental Health America